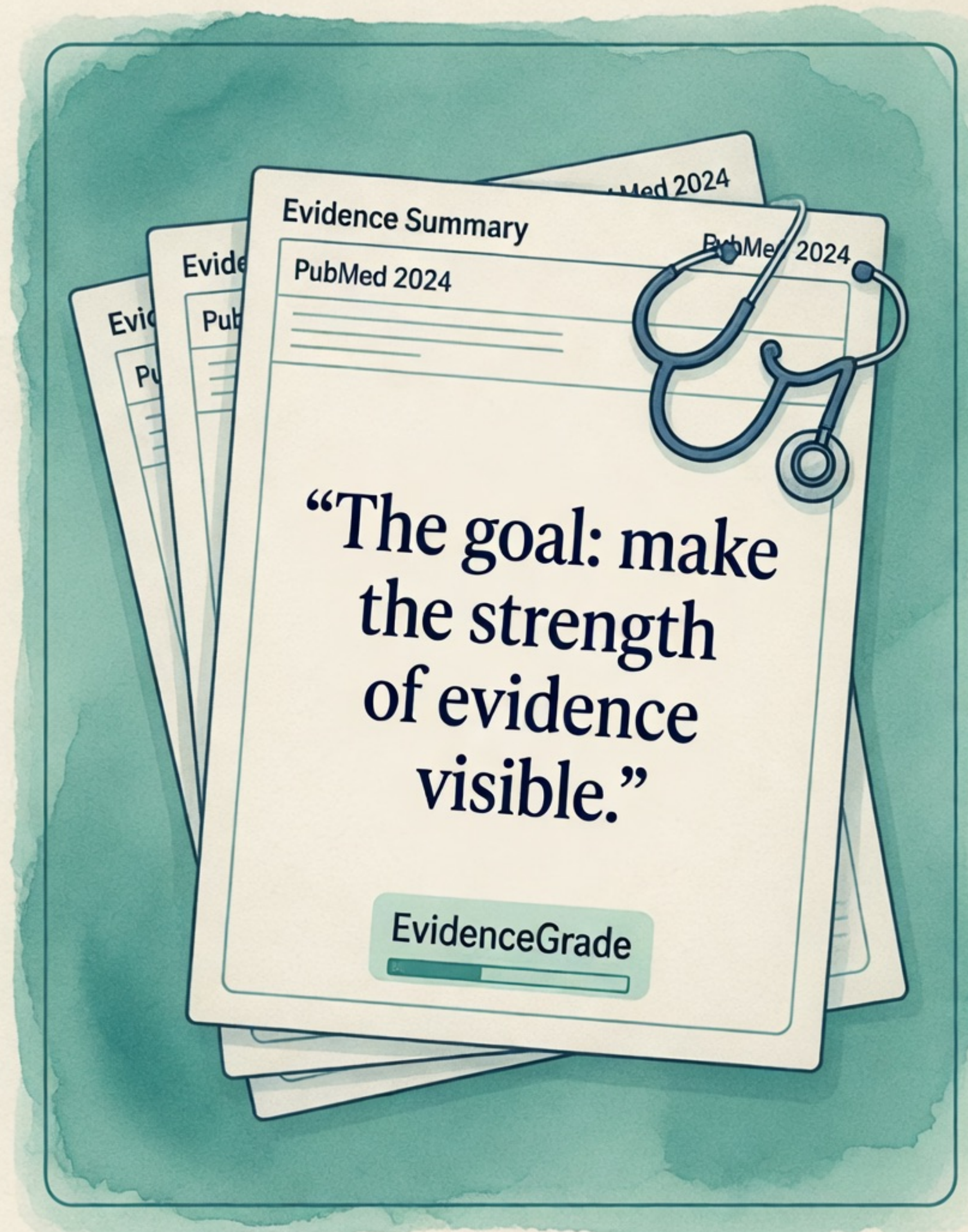
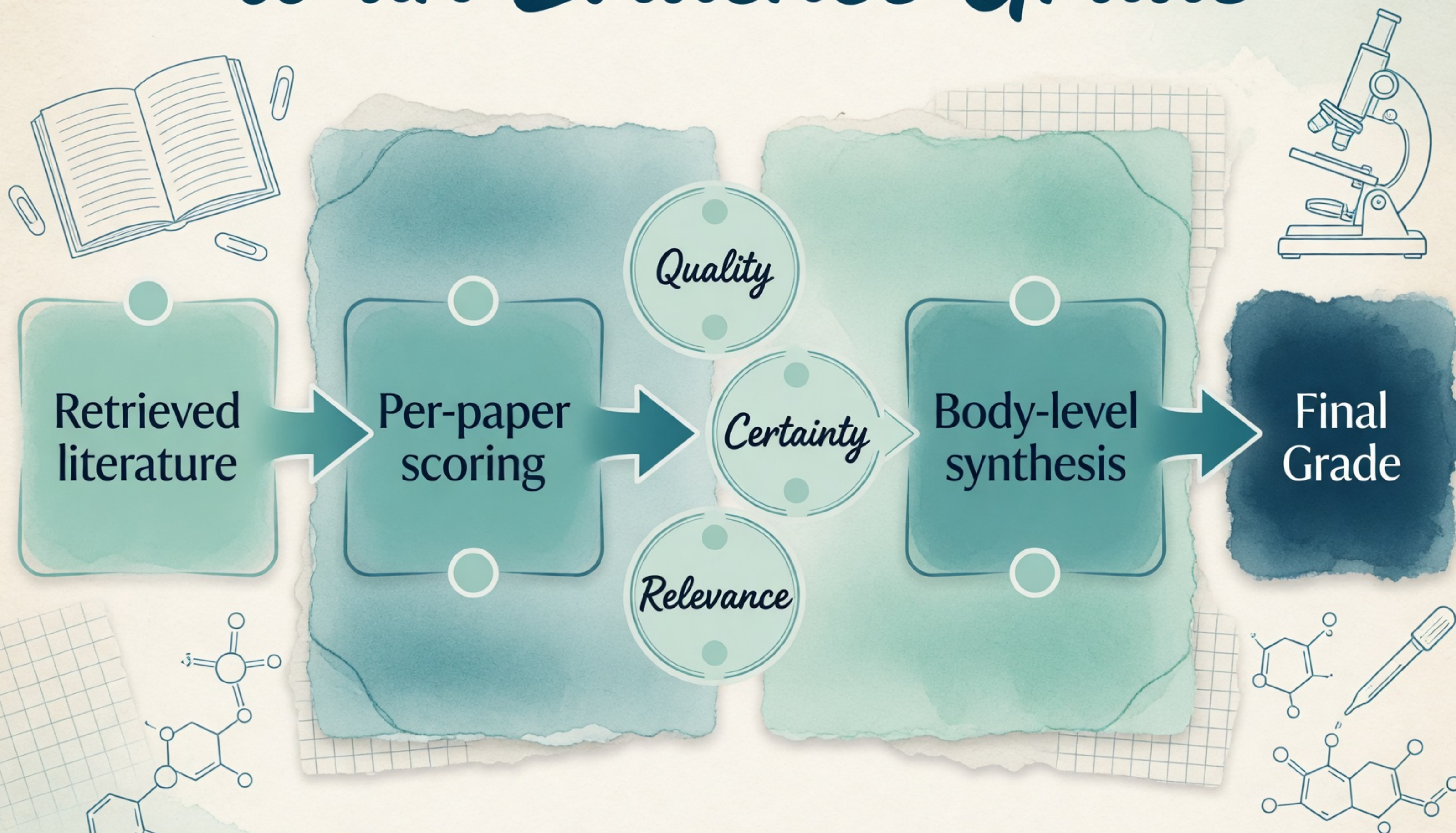


How OpenEvidence EvidenceGrade Approaches Clinical AI Confidence

- Real-time strength-of-evidence assessment
- GRADE-inspired, not a systematic review
- Grades A–D, +/- modifiers, and U
- Focus: the evidence behind a specific answer



“From Retrieved Papers to an Evidence Grade”



The grade reflects the body of evidence — not citation count.

What Each Evidence Grade Means



A

Strong
evidence

B

Moderate
evidence

C

Limited
evidence

D

Minimal
evidence

U

Unable
to grade

U is not failure. It is an explicit signal that evidence cannot be responsibly graded.

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“Where a Real-Time Grade Can Fall Short”

● Retrieval can miss pivotal studies

● One letter can hide different sources of uncertainty

● Relevance depends on patient, setting and outcome

● Independent clinical-methodology calibration is essential

“Useful decision support is not the same as a final clinical verdict.”



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Stay Informed on Evidence-Aware Clinical AI

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- Ask: does the AI show its evidence — and its uncertainty?



The future of clinical AI
is not greater certainty.
It is more visible uncertainty.

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